Case study: Talia

Condition: **Cerebral palsy – spastic diplegia (GMFCS III)**

15-year-old girl · School-based occupational therapy, state high school

Talia Rossi is a 15-year-old girl with **spastic diplegic cerebral palsy** who is in Year 10 at a large state high school on the Sunshine Coast. She lives with her mum, Gina, and her younger brother. Talia is classified at GMFCS level III. She walks with a posterior walker inside the school buildings and uses a manual wheelchair for longer distances. A teacher aide supports her for part of each school day. Talia moved to this school in Year 7, after a difficult time at her previous school, where she was often left out of activities.

Talia's school file describes her as "a capable, motivated student" who is achieving As and Bs. Her physiotherapist's letter from August states that she is walking safely with her walker and has had "no falls this year". Her attendance has been excellent all term.

You have been asked to see Talia during a free period to finalise her exam adjustments for the end-of-year exams, including extra time, rest breaks and a laptop with speech-to-text software. The school's adjustment form is due to the deputy principal this afternoon. Talia arrives chatty and joking, telling you about a video she made with her friends and teasing you about your handwriting.

As you set up the speech-to-text software, Talia keeps rubbing her forehead and squinting at the screen. She asks you to turn the brightness down and says she has "the worst headache", which has been "getting worse since last night". She mentions that she vomited before school, "but it was probably the pie I had for dinner". She yawns repeatedly and twice rests her head on the desk. When you ask what time her English exam starts, she says it is tomorrow, then frowns and corrects herself: it is today, in the next period.

When you move on to the section of the form about toilet access, Talia goes quiet. She explains that the accessible toilet near her classrooms is used to store sports equipment, so she uses a standard cubicle and holds onto the basin to turn around. "Yesterday I slipped in there and smacked my head on the basin," she says. "It's fine. I didn't tell anyone." When she moves her hair aside, you see a swollen, tender area above her left ear. She adds that she does not drink anything at school "so I don't have to go in there", and you notice her drink bottle is still full.

Talia then grabs your arm. "Please don't tell my mum," she says. "She'll make me change schools again, and I'll lose all my friends."

There is no clear documentation regarding the fall, whether any first aid was given or anyone checked on her afterwards, whether she takes any regular medication, or how much she has eaten and drunk today. There is no record of whether the toilet access issue has been raised with the school before.

As you are speaking, Mrs Nguyen, Talia's teacher aide, comes in holding a risk assessment form for next month's Year 10 camp. She asks whether you could "quickly sign off on the camp bits" and then says, "The bell's about to go. Talia's exam starts in five minutes, so I'll walk her over now." Talia pulls her walker towards her and stands up, swaying slightly. "See? I'm fine," she says. "I just need to get through this exam."

You are the occupational therapy student reviewing Talia's exam adjustments and your clinical supervisor is not currently in the room.

Present Talia's case to your clinical supervisor using the ISBAR framework. Identify the immediate concerns and explain what you would prioritise in responding to the situation.

ISBAR planning sheet

Use your 10 minutes of preparation to plan. Then speak for 7 minutes to your “supervisor”.

I Introduction

≈ 20 s

Who you are, your role, who the client is and why you need to talk now.

S Situation

≈ 40 s

A brief but complete summary of what is happening and the immediate concern.

B Background

≈ 1 min

Relevant history: the primary problem, medical and social history, medications/allergies, interventions so far and OT to date. Name missing information rather than assuming it.

A Assessment

≈ 2 min 20 s

Your interpretation. Case finding → what it may mean → consequence for safety and occupations → priority. Separate facts, interpretation, gaps and proposed assessment. What needs escalation?

R Recommendation

≈ 2 min 40 s

What should happen next, in priority order. Action → reason → who → review. Immediate safety, escalation, MDT input, one short-term and one longer-term OT assessment, intervention, referrals and follow-up.

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